

Emergency Admission & Escalation Procedures

Admitting an Emergency Patient

- Step 1: Patient is triaged and assigned an ESI level (see emergency_triage.pdf).
- Step 2: The attending ED physician performs an assessment and orders any needed labs or imaging.
- Step 3: If admission is required, the physician contacts the relevant department (e.g., Cardiology, General Surgery) for a bed and hand-over.
- Step 4: Nursing staff complete the admission checklist, including allergy bands, consent forms, and initial nursing assessment.
- Step 5: The patient is transferred to the ward or ICU with a full handover summary.

Escalation of Care

- Any patient who deteriorates (drop in oxygen saturation, altered consciousness, unstable vitals) triggers a Rapid Response Team (RRT) call.
- The RRT includes a senior physician, an ICU nurse, and a respiratory therapist, and must respond within 5 minutes of being paged.
- All RRT activations are logged and reviewed weekly by the Patient Safety Committee.

Inter-Hospital Transfers

- Patients requiring a level of care not available on-site (e.g., specialized neurosurgery) are stabilized first, then transferred with a full medical summary and consent.
- A transfer is never initiated until the receiving facility has confirmed bed availability.